

03-icd-implantation-policy

Payer Prior Authorization Policy: Implantable Cardioverter-Defibrillator (ICD) — Primary Prevention

CPT Code: 33249 **Applicable ICD-10 codes:** I50.22 (Chronic systolic heart failure), I42.0 (Dilated cardiomyopathy), I25.5 (Ischemic cardiomyopathy) **Policy Number:** CARD-ICD-021
Effective Date: 2026-01-01

Purpose

Defines clinical criteria for prior authorization of implantable cardioverter-defibrillator (ICD) placement for PRIMARY prevention of sudden cardiac death (not for members with a prior documented sustained ventricular arrhythmia or cardiac arrest, which fall under secondary prevention policy CARD-ICD-022).

Required Supporting Documentation

1. **Left ventricular ejection fraction (LVEF) assessment** — An echocardiogram, nuclear (MUGA), or cardiac MRI report performed within the 3 months preceding the request, documenting $LVEF \leq 35\%$.
2. **Guideline-directed medical therapy (GDMT) trial** — Clinical/cardiology notes documenting at least 3 consecutive months of optimized guideline-directed medical therapy, which must include an ACE inhibitor, ARB, or ARNI AND a beta-blocker (with specific drug names, doses, and dates), with a note that the LVEF above was reassessed AFTER this trial period, not before it.
3. **NYHA functional class** — Documented New York Heart Association (NYHA) functional classification of II or III at the time of evaluation. Class I or IV (non-transplant/non-LVAD candidate) does not meet criteria under this policy.
4. **Waiting period documentation** — For ischemic cardiomyopathy: documentation that at least 40 days have elapsed since the most recent myocardial infarction. For non-ischemic cardiomyopathy: documentation that at least 3 months have elapsed since diagnosis and initiation of GDMT. The specific qualifying date (MI date or diagnosis date) must be stated.
5. **Reasonable survival expectation** — A note from the treating cardiologist attesting to a reasonable expectation of meaningful survival greater than 1 year with good functional status.
6. **Exclusion criteria addressed** — Documentation that the member has NOT had a revascularization procedure (CABG or PCI) within the preceding 3 months without a post-procedure LVEF reassessment, and is not within 40 days of an MI without the waiting period above being satisfied.
7. **Cardiologist attestation** — A signed statement of medical necessity from a board-certified cardiologist or electrophysiologist (not a general internist or PCP alone).

Exclusions

- Members with a documented prior sustained ventricular tachycardia, ventricular fibrillation, or resuscitated cardiac arrest are excluded from this policy and must be evaluated under the secondary-prevention policy (CARD-ICD-022), which has different, less restrictive waiting-period criteria.
- Subcutaneous ICD (S-ICD) requests are reviewed under a separate device-specific addendum.